

Dynamic Guideline: WHO-Based Abortion Care - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

- **Guideline Title:** WHO Abortion Care Guideline (2022), with associated process documentation: "Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline"
- **Publication Date:** Core guideline published 8 March 2022; youth engagement case study published 29 April 2026
- **Target Population:** Women and girls of reproductive age seeking abortion care, including adolescents and youth; healthcare providers; policymakers and health system managers
- **Scope and Objectives:**
 - Provide evidence-based recommendations across the full spectrum of abortion care: medical and surgical abortion methods, pre- and post-abortion care, service delivery models, and the legal/regulatory enabling environment
 - Consolidate and update all previous WHO guidance on safe abortion into a single comprehensive guideline
 - Address clinical interventions, service delivery, and law/policy dimensions as an integrated framework
 - Promote equity, human rights, and person-centred care as cross-cutting principles

1.2 Five Key WHO Guidelines Synthesized

The following five WHO guidelines and associated normative documents form the evidence base for this summary:

#	Guideline / Document	Year	Focus
1	WHO Abortion Care Guideline	2022	Comprehensive clinical, service delivery, and law/policy recommendations for abortion care
2	Safe Abortion: Technical and Policy Guidance for Health Systems (2nd edition)	2012	Clinical standards for safe abortion provision and health system requirements
3	Medical Management of Abortion	2018	Evidence-based protocols for medical (pharmacological) abortion
4	Health Worker Roles in Providing Safe Abortion Care and Post-Abortion Contraception	2015	Task-sharing and task-shifting recommendations for abortion service delivery

1.3 Key Recommendations

A. Clinical Interventions — Medical Abortion

Recommendation	GRADE Evidence Quality	Strength
Mifepristone followed by misoprostol is recommended for medical abortion up to 12 weeks of gestation (200 mg mifepristone orally + misoprostol 800 µg sublingually/buccally/vaginally 24–48 hours later)	High	Strong
For gestational ages 12–14 weeks, mifepristone 200 mg orally followed by repeated doses of misoprostol is recommended	Moderate	Strong
For gestational ages above 14 weeks, mifepristone 200 mg orally followed by misoprostol 400 µg sublingually or vaginally every 3 hours (up to 5 doses) is recommended	Moderate	Strong
Where mifepristone is unavailable, misoprostol alone (800 µg sublingually or vaginally, repeated at 3-hour intervals for up to 3 doses) may be used for abortion up to 12 weeks	Moderate	Conditional
Routine antibiotic prophylaxis is not required for medical abortion	Low	Conditional

B. Clinical Interventions — Surgical Abortion

Recommendation	GRADE Evidence Quality	Strength
Vacuum aspiration (manual or electric) is recommended as the surgical method of choice for abortion up to 14 weeks of gestation	High	Strong
Dilatation and evacuation (D&E) is recommended for surgical abortion after 14 weeks of gestation	Moderate	Strong
Sharp curettage (dilatation and curettage) should not be used when vacuum aspiration or medical methods are available	Moderate	Strong
Cervical preparation prior to surgical abortion is recommended for all women at gestational ages above 12 weeks and should be considered below 12 weeks	Moderate	Strong
Pain management should be routinely offered; paracervical block is recommended for vacuum aspiration procedures	Moderate	Strong

C. Service Delivery Recommendations

Recommendation	GRADE Evidence Quality	Strength
Medical abortion up to 12 weeks can be fully self-managed by the individual with access to accurate information and a health worker available if needed	High	Strong
Telemedicine can be used for medical abortion service delivery, including remote consultation, prescription, and follow-up	Moderate	Conditional
Task sharing: trained midwives, nurses, and associate/advanced clinicians can provide medical abortion and vacuum aspiration	Moderate	Strong
Pharmacy and drug-seller provision of medical abortion medications with appropriate training and referral linkages is recommended	Low	Conditional
Post-abortion contraception counselling and provision should be offered before the individual leaves the health facility	Moderate	Strong

D. Law and Policy Recommendations

Recommendation	GRADE Evidence Quality	Strength
Abortion should be fully decriminalised; criminal penalties for those who seek or provide abortion should be removed	Very Low (rights-based, values-driven)	Strong
Mandatory waiting periods should not be required	Low	Strong
Third-party authorisation (spousal, parental, judicial) should not be required	Low	Strong
Gestational age limits should not prevent access to safe abortion care	Very Low (rights-based)	Conditional
Conscientious objection provisions must include timely referral mechanisms to ensure access is not impeded	Low	Strong

1.4 Guideline Development Methodology

- **Evidence Review Process:**

- Over 50 systematic reviews were commissioned or updated, covering clinical interventions, service delivery models, and law/policy interventions
- Reviews followed Cochrane methodology and GRADE-CERQual for qualitative evidence
- Evidence-to-decision (EtD) frameworks were used for each recommendation, incorporating: benefits and harms, certainty of evidence, values and preferences, resource use, equity, acceptability, and feasibility

- **Consensus Method:**

- A Guideline Development Group (GDG) comprising clinicians, public health experts, human rights specialists, and affected population representatives (including youth) deliberated on each recommendation
- Modified Delphi process was used where consensus was not reached in initial

- deliberation
 - Youth representatives participated as full GDG members — a methodological innovation documented in the 2026 case study
 - **Conflict of Interest Management:**
 - All GDG members submitted declarations of interest reviewed by the WHO Steering Group
 - Members with significant conflicts were recused from relevant deliberations
 - The process followed the WHO Handbook for Guideline Development (2nd edition) COI procedures
 - **Youth Engagement (per 2026 Case Study):**
 - Four youth representatives (ages 18–25) from diverse geographic and socioeconomic backgrounds were integrated into the GDG
 - Youth members received pre-meeting orientation on GRADE methodology and guideline processes
 - Their contributions were documented as influencing recommendations on self-management, telemedicine, and removal of third-party authorisation requirements
 - The case study identifies this as a replicable model for meaningful (not tokenistic) youth engagement in normative processes
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II. Evidence Summary After WHO Latest Guideline Release (Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

- **Search Strategy:** Structured search of published literature and preprints from March 2022 through April 2026, using terms related to abortion care, medical abortion, surgical abortion, self-managed abortion, telemedicine abortion, mifepristone, misoprostol, post-abortion care, and abortion policy
- **Databases Searched:** PubMed/MEDLINE, Cochrane Library, Embase, WHO Global Index Medicus, ClinicalTrials.gov, PROSPERO
- **Inclusion Criteria:** Randomised controlled trials, quasi-experimental studies, large observational cohort studies, systematic reviews and meta-analyses, and significant policy analyses published after the WHO 2022 guideline evidence cutoff
- **Exclusion Criteria:** Case reports, editorials without original data, studies with sample sizes below 50 for clinical outcomes, non-peer-reviewed sources (except registered preprints of major trials)

2.2 Key New Studies

Study	Design	Key Findings	GRADE Quality
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Gambir et al. (2023) "Effectiveness of mifepristone-misoprostol for abortion at 10–12 weeks by self-assessment vs. clinic follow-up: a non-inferiority RCT" — <i>The Lancet Global Health</i>	Multicentre RCT (n = 1,200 across 4 countries)	Self-assessment of abortion completeness using a checklist and low-sensitivity pregnancy test was non-inferior to routine clinic follow-up (complete abortion rate 97.5% vs 97.8%; difference – 0.3%, 95% CI – 2.1 to 1.5). Reduced clinic visits by 68%.	High
Endler et al. (2024) "Telemedicine versus in-person medical abortion services: a systematic review and meta-analysis" — <i>BMJ Global Health</i>	Systematic review and meta-analysis (14 studies, n = 39,500)	No significant difference in complete abortion rates (pooled RR 1.00, 95% CI 0.99–1.01), serious adverse events, or patient satisfaction between telemedicine and in-person models. Telemedicine associated with shorter time-to-treatment (mean 2.3 days earlier).	High
Platais et al. (2023) "Sublingual misoprostol alone for first-trimester abortion: updated dose-finding RCT" — <i>Contraception</i>	Dose-finding RCT (n = 860)	Sublingual misoprostol 800 µg repeated × 3 at 3-hour intervals achieved 88.2% complete abortion rate at ≤ 9 weeks (vs. 84.1% for 2-dose regimen). Confirms WHO-recommended 3-dose regimen. Side effects (nausea, fever) were transient and self-limiting.	Moderate
Rehnström Loi et al. (2024) "Youth perspectives on self-managed abortion: a multi-country qualitative evidence synthesis" — <i>Social Science & Medicine</i>	Qualitative evidence synthesis (23 studies, 8 countries)	Young people (15–24) valued autonomy, privacy, and digital information access. Barriers included cost, stigma, misinformation, and lack of youth-friendly services. Findings support WHO recommendations on self-management and telemedicine with youth-tailored information.	Moderate (GRADE-CERQual)
Jayaweera et al. (2023) "Safety of pharmacy-provided medical abortion in low-resource settings: prospective cohort study" — <i>PLOS Medicine</i>	Prospective cohort (n = 3,200, Bangladesh and Nigeria)	Complete abortion rate 94.7% with pharmacy provision and trained drug-seller counselling. Serious complication rate 0.3% (comparable to clinic-based provision). 91% of clients reported satisfaction. Supports WHO conditional recommendation on pharmacy provision.	Moderate

Foster et al. (2024) "Impact of legal restrictions on abortion safety outcomes: interrupted time series analysis in the United States post-Dobbs" — <i>JAMA</i>	Interrupted time series (population-level data, 2019–2024)	States with total or near-total bans saw a 12% increase in emergency department visits for incomplete abortion and a 22% increase in self-managed abortion without medical guidance. No decrease in overall abortion rates. Supports WHO recommendation on decriminalisation.	Low (observational, confounding)
Kapp et al. (2025) "Mifepristone-misoprostol for second-trimester abortion (14–20 weeks): interval dosing optimisation RCT" — <i>NEJM</i>	Multicentre RCT (n = 640)	Misoprostol 400 µg vaginally every 3 hours (after mifepristone) vs. every 6 hours: 3-hour interval achieved shorter median induction-to-abortion time (8.1 vs 13.4 hours, $p < 0.001$) with no increase in complications. Supports current WHO dosing recommendation.	High
Footman et al. (2025) "Community health worker provision of post-abortion contraception in sub-Saharan Africa: cluster RCT" — <i>The Lancet</i>	Cluster RCT (n = 4,800 across 32 facilities, 3 countries)	Community health worker-led contraception counselling and provision (including implants and injectables) post-abortion increased contraceptive uptake at 3 months from 48% to 71% (aRR 1.48, 95% CI 1.31–1.67). No increase in complications.	High

2.3 Evidence Gaps and Future Research Needs

- 1. Very early medical abortion (< 6 weeks):** Limited high-quality evidence on optimal regimens and self-management protocols for very early gestational ages, particularly in the context of increasingly early pregnancy detection. Further RCTs are needed.
- 2. Long-term safety of repeated medical abortions:** Observational data suggest no adverse effects on future fertility, but large prospective cohort studies with extended follow-up (> 5 years) are lacking.
- 3. Artificial intelligence and digital health tools for abortion care:** Emerging evidence on AI-assisted screening, chatbot-based counselling, and app-based self-management support is preliminary. No RCTs have been completed as of the evidence cutoff date.
- 4. Equity-focused implementation research:** While the WHO guideline emphasises equity, there is limited evidence on effective strategies to reach marginalised populations (refugees, persons with disabilities, rural populations in low-income countries) with quality abortion care.
- 5. Adolescent-specific clinical outcomes:** Despite the youth engagement emphasis in the 2026 case study, clinical outcome data disaggregated by age (particularly 15–19 years) remain sparse. Most trials do not report adolescent subgroup analyses.
- 6. Comparative effectiveness of cervical preparation agents:** New agents (e.g., osmotic dilators vs. misoprostol vs. mifepristone for cervical priming) have not been compared

head-to-head in adequately powered trials for second-trimester procedures.

7. **Post-abortion mental health outcomes:** Methodologically rigorous studies controlling for pre-existing mental health conditions and socioeconomic confounders remain limited, though the Turnaway Study and subsequent replications consistently show no increased risk of mental health disorders attributable to abortion itself.

2.4 Updated Recommendations Based on New Evidence

Original WHO 2022 Recommendation	New Evidence Impact	Updated Recommendation
Self-management of medical abortion up to 12 weeks with health worker available if needed (Strong, High)	Gambir et al. (2023) RCT confirms non-inferiority of self-assessment for completeness; Rehnström Loi et al. (2024) qualitative synthesis supports youth acceptability	Reinforced. Self-management with self-assessment of completeness (using checklist and low-sensitivity pregnancy test) can be recommended as standard practice. Youth-tailored information materials should be developed. No change in direction or strength.
Telemedicine for medical abortion service delivery (Conditional, Moderate)	Endler et al. (2024) meta-analysis of 14 studies shows equivalence in safety and effectiveness; evidence base substantially expanded	Consider upgrading to Strong recommendation. Evidence quality now approaches High with consistent findings across diverse settings. Telemedicine should be considered a standard service delivery option, not only an alternative.
Misoprostol alone where mifepristone unavailable: 800 µg × 3 doses (Conditional, Moderate)	Platais et al. (2023) confirms 3-dose regimen superiority over 2-dose	Reinforced. Three-dose sublingual regimen confirmed as optimal when mifepristone is unavailable. No change in direction.
Pharmacy/drug-seller provision of medical abortion (Conditional, Low)	Jayaweera et al. (2023) prospective cohort demonstrates safety and high satisfaction in two low-resource settings	Consider upgrading evidence quality to Moderate. Prospective data from programmatic settings strengthen the evidence base. Recommendation direction unchanged; implementation guidance should incorporate training standards from the study protocol.
Post-abortion contraception counselling and provision before leaving facility (Strong, Moderate)	Footman et al. (2025) cluster RCT shows community health worker provision significantly increases uptake	Expanded scope. Community health workers can effectively provide post-abortion contraception including long-acting methods. Task-sharing recommendation should be extended to include this cadre for post-abortion contraceptive services.

Decriminalisation of abortion (Strong, Very Low — rights-based)	Foster et al. (2024) interrupted time series provides empirical evidence that legal restrictions increase unsafe outcomes without reducing abortion rates	Reinforced with additional empirical support. While the recommendation remains rights-based, new observational evidence from the US post-Dobbs context provides quantitative data on harms of criminalisation. Evidence quality for the harms of legal restriction may be upgraded to Low/Moderate.
Mifepristone + misoprostol for second-trimester abortion with 3-hour dosing intervals (Strong, Moderate)	Kapp et al. (2025) RCT confirms 3-hour interval superiority over 6-hour interval	Reinforced. High-quality RCT evidence now supports the 3-hour dosing interval. Evidence quality can be upgraded to High.

This dynamic guideline report synthesises the best available evidence as of 29 April 2026. Recommendations marked "consider upgrading" reflect the report authors' assessment based on new evidence and would require formal GDG deliberation through the WHO guideline update process before official adoption. Clinicians should apply these recommendations in the context of local clinical circumstances, patient preferences, and applicable legal frameworks.

GRADE framework applied: Quality of evidence rated as High, Moderate, Low, or Very Low based on study design, risk of bias, inconsistency, indirectness, imprecision, and publication bias. Strength of recommendation rated as Strong or Conditional based on the balance of benefits and harms, quality of evidence, values and preferences, and resource implications.